

GLOBAL HEALTH DATA • IHME / GBD 2023

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Audience: Public Health Policy Analysts & Program Planners

Data: Our World in Data · Dattani et al. (2023)

Scope: World, 2023

Hundreds of Millions Affected

1 in 3

Women will experience major depression in their lifetime.

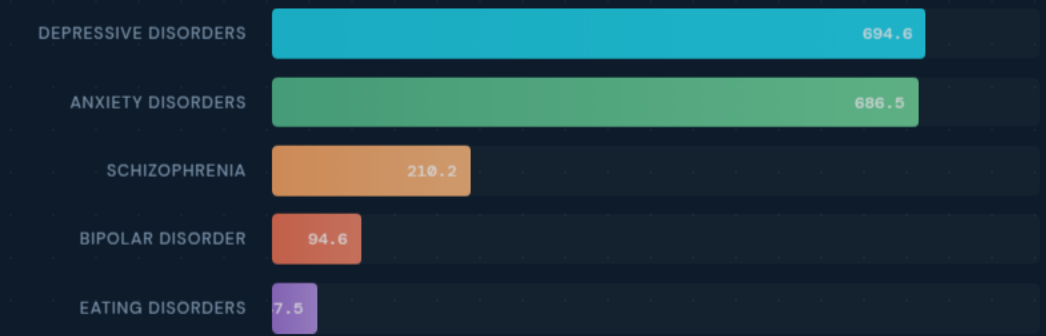
1 in 5

Men will experience major depression in their lifetime.



Depression & Anxiety Dominate Global Burden

DALYs per 100,000 people — combining years of life lost and years lived with disability.
World, 2023.



Source: IHME, Global Burden of Disease (2023) via Our World in Data · Dattani, Rodés-Guirao, Ritchie & Roser



Nearly Equal Disease Burden: Depression & Anxiety

The two most prevalent mental health categories differ by less than 1.2% in DALYs — a striking parity with major policy implications.

694.6

DALYs / 100K

686.5

DALYs / 100K

Depressive
Disorders

Anxiety
Disorders

Together, depressive and anxiety disorders account for the **vast majority of the global mental health disease burden** — dwarfing all other categories combined. Both demand equal priority in screening, prevention, and treatment programs.



Lower Prevalence — But Severe Individual Impact

Schizophrenia and bipolar disorder carry a disproportionately high burden per person relative to their population-level prevalence.

SCHIZOPHRENIA

210.2

DALYs per 100,000

Severe disability associated with the condition drives a high DALY burden despite affecting a smaller share of the population. Specialized, sustained care is essential.

BIPOLAR DISORDER

94.6

DALYs per 100,000

Episodes of mania and depression significantly impair functioning. While the aggregate DALY rate is lower, individual impact and need for long-term management remain substantial.

REFERENCE: DEPRESSION

694.6

DALYs per 100,000

Provided for scale comparison. Schizophrenia is roughly 3× lower; bipolar is roughly 7× lower in aggregate DALYs vs. depressive disorders.

EATING DISORDERS

47.5

DALYs per 100,000

The smallest share among these categories, yet meaningful in absolute terms — particularly among younger populations — and often underdiagnosed.

Ratio bars show each condition's DALYs relative to depressive disorders (maximum = 100%). Source: IHME GBD 2023.



Treatment Is Available — But Often Lacking or Poor Quality

Mental illnesses are treatable. Yet across all income settings, a significant gap persists between need and actual care received.

Despite the availability of **effective treatments**, most people with mental health conditions do not receive adequate care — especially in low- and middle-income countries. The true burden is likely higher than reported figures suggest due to systematic underreporting.



Stigma

Many people are uncomfortable disclosing symptoms to healthcare professionals or even people they know, suppressing both help-seeking and accurate prevalence data.



Access

Treatment is often lacking, especially in low- and middle-income countries where mental health infrastructure and trained providers are scarce.



Quality

Even where treatment exists, it may be of poor quality — outdated protocols, insufficient follow-up, or inadequate dosing of evidence-based interventions.



Underreporting

Discomfort with disclosure makes true prevalence estimates difficult. Actual burden may substantially exceed reported figures — meaning the treatment gap is likely larger than data shows.

How People Cope: Medication, Faith, and Family

Global survey data shows wide variation in how people deal with anxiety or depression — shaped by culture, income, and healthcare context.



Prescribed Medication

More common in higher-income settings with accessible healthcare. Antidepressants and anxiolytics form the backbone of pharmacological treatment.

Higher-income settings



Religious & Spiritual Activities

Prevalent across many regions worldwide. Prayer, religious community, and spiritual practice are widely reported coping mechanisms, particularly in lower-income contexts.

Widespread globally



Talking to Friends or Family

A universally reported coping mechanism — informal social support provides an accessible buffer against untreated conditions across all income levels.

Universal

Policy implication: The diversity of coping strategies underscores that responses are shaped by cultural, economic, and healthcare system factors. Effective policy must account for these differences — a one-size-fits-all approach will fail to reach those who need it most.



Key Takeaways: Closing the Treatment Gap

Evidence-based priorities for mental health policy analysts and program planners.

- 01 Depression and anxiety dominate the global burden** — each generating ~690 DALYs per 100,000, together accounting for the overwhelming majority of mental health disease burden. Both require equal policy priority.
- 02 Schizophrenia and bipolar carry disproportionate individual burden** — despite lower population-level DALYs (210.2 and 94.6 respectively), individual severity and disability are high. Specialized, sustained care must be preserved and expanded.
- 03 Treatment exists but access and quality remain inadequate** — especially in low- and middle-income countries. Scaling evidence-based interventions is both feasible and urgent.
- 04 Stigma suppresses both help-seeking and accurate data** — the true prevalence and burden are likely higher than reported figures. Anti-stigma campaigns are a prerequisite for effective policy.
- 05 Scale interventions for common disorders; maintain specialized care for severe conditions** — coping strategies vary widely, requiring culturally sensitive, tiered service models.

Data: IHME, Global Burden of Disease (2023) · Our World in Data · Dattani, Rodés-Guirao, Ritchie & Roser (2023)

